BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

Check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Blood / kidney / liver	full blood count (FBC), renal and liver profile	FBC before Days 1, 8 and 15	Renal and liver tests before each cycle	Review unresolved abnormalities	Individual clinical plan
Heart / breathing	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/breathing symptoms	Clinical indication
Cancer review	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks	Review previous treatment	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00749 v3 · official regimen · NICE NG101 · bone health and follow-up · ASCO · biomarkers in metastatic breast cancer · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

Gemcitabine 800 mg/m² · 28-day schedule: treatment and checks

Advanced breast cancer after two or more treatment lines

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00749	Confirm exact regimen	Gemcitabine 800 mg/m² · 28-day schedule	Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00749	Advanced breast cancer after two or more treatment lines	Follow the multidisciplinary team (MDT) sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00749	FBC, renal and liver profile	FBC before Days 1, 8 and 15	Renal and liver tests before each cycle	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00749	Review symptoms and prior treatment	Investigate clinical concerns	No routine HER2-style echo schedule	Review new chest/breathing symptoms	Clinical indication
Cancer review NCCP 00749 · METASTATIC · MARKERS	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Setting-specific review
Other checks NCCP 00749 · BONE	Review previous treatment	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1 Baseline review

Before treatment

Advanced breast cancer after two or more treatment lines. There is no routine three-month echo schedule for CMF or gemcitabine alone. Review prior treatment and cardiac risk; new chest pain or breathing symptoms need urgent assessment.

Ask / **act:** Confirm source eligibility and results.

Sources: NCCP 00749

2 Treatment-day check

Before each planned dose

Full blood count before every planned treatment on Days 1, 8 and 15; kidney and liver tests before each cycle. Initial eligibility, new-cycle and later-day thresholds are different.

Ask / **act:** Confirm active medicines and whether a dose is held.

Sources: NCCP 00749

3 Support plan

At every phase or day change

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

Ask / **act:** Choose the matching support script; do not merge phases.

Sources: NCCP 00749

4 Response and follow-up

Agreed review points

For metastatic disease, record the disease sites and agree the scan type and review interval before treatment. Review response, symptoms and toxicity together; worsening symptoms may need earlier assessment.

Ask / **act:** Review symptoms, results and patient priorities together.

Sources: NCCP 00749 · METASTATIC · MARKERS

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.18